Outlook

FW: Policy Committee Support Paper.pdf

From Pope, Clinton (IHS/PHX/PIMC) <Clinton.Pope@ihs.gov>
Date Fri 01-Mar-24 09:03
To Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Cc McCabe, Endreya (IHS/PHX) <Endreya.McCabe@ihs.gov>

1 attachment (355 KB)
Policy Committee Support Paper.pdf;

Kevin,

BLUF: Let's (you, Endreya, and me) meet to strategize on organizational policy management. If needed, let's invite others. Let's meet regularly for a few months to develop a good strategic plan. Let's own this and not appear to be floundering with this to other leaders. We have the authority and skills to fix this ourselves. I have no doubt that I have the full support of Deb and Executive leadership.

I hope you know I fully support the work that needs done in policy management, as does other leadership at PIMC.

Please allow me to provide some feedback, because I want to improve communication and get better results with our organizational approach to policy management. Regarding recent communications:

- It feels like scare tactics, meaning that it's soliciting action by appealing to fear with words and phrases like, "significant compliance issues," "threatening patient safety," "critical compliance failures," "widespread violations," "jeopardize patient well-being," "undermine PIMC's mission." I prefer a more transparent, data-driven approach to seeing things as they really are followed by honest focus on techniques that will move us in the direction we need to go.
- Most of it feels like it's written by ChatGPT, lacking a certain personal or timely touch.
- It feels accusatory as if it places blame for not doing enough. I ask that we focus on designing systems and processes that will lead to the desired improvements.
- Way too many words. Anytime you're communicating with senior leadership, I recommend using tools like a simple SBAR or BLUF, that is highlighting a problem and offering solutions or making a basic request/recommendation.
- Not using chain of command. Endreya and I are supportive of the work that needs done. Please work with us and with committee structures to accomplish the work. There is and should always be an open door policy that allows you to reach directly to the CEO if needed, but this is something that should be done sparingly or when other avenues are exhausted. By sending these types of emails to Deb, it communicates that Endreya and I are failing at our jobs, and if that's how you feel, I'd appreciate your feedback.

Things move very slowly in the government, which is often very frustrating to me and others, but we need to exercise patience. There are competing and shifting priorities that can't all be done at once. These policy issues will likely take years to improve and will never be perfect, and that is okay as long as we are making improvements. Our policies are as amazing as they are imperfect. Let's take a continuous improvement approach with incremental improvements over time.

Very Respectfully,

Clinton D. Pope, MSIE, CPHQ

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From: Ward Lund, Debra L. (IHS/PHX) <Debra.Wardlund@ihs.gov>
Sent: Thursday, February 29, 2024 5:24 PM
To: Pope, Clinton (IHS/PHX/PIMC) <Clinton.Pope@ihs.gov>; Tillman, Laura (IHS/PHX) <Laura.Tillman@ihs.gov>
Subject: FW: Policy Committee Support Paper.pdf

Let's find some time to review this next week…

Thanks,

Deb

From: Ward Lund, Debra L. (IHS/PHX)
Sent: Thursday, February 29, 2024 5:23 PM
To: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Subject: RE: Policy Committee Support Paper.pdf

Thank you. I will follow up on this and let you know if I may need additional information.

Deb

From: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Sent: Thursday, February 29, 2024 5:21 PM
To: Ward Lund, Debra L. (IHS/PHX) <Debra.Wardlund@ihs.gov>
Subject: Re: Policy Committee Support Paper.pdf

(PIMC) has significant compliance issues threatening patient safety and operational efficiency. The mock survey highlighted critical violations in behavioral health protocols, regulatory standards, and procedures. Immediate action is required; I recommend establishing a dedicated policy committee or, at the very least, revamping how current committees approve policies. The committees should focus on evidence-based policy development, stakeholder engagement, and regular compliance assessments to ensure safe, culturally sensitive care for American Indian/Alaska Native patients and align with the center's mission. We have policies referencing materials from 2001 that are not evidence-based or safe.

V/r

Hon. Kevin D. Turnage, MSN Ed., RN, AMB-BC™, NE-BC®, NHDP-BC™, NPD-BC™
Policy Officer (Acting), COR II,
Phoenix Indian Medical Center
Cell: 602-647-9126

"The single greatest impediment to error prevention in the medical industry is that we punish people for making mistakes." Dr. Lucian Leape

I am not authorized to obligate the government for any work, service or product. Please do not perform any work that will require payment without a valid contract, issued by a government Contracting Officer. Changes to a contract can only be made by the contracting officer. Contractors are reminded to promptly notify the contracting officer of any conduct that may be regarded as a change to the contract in accordance with FAR52-243-7

From: Ward Lund, Debra L. (IHS/PHX) <Debra.Wardlund@ihs.gov>
Date: Thursday, February 29, 2024 at 17:08
To: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Subject: RE: Policy Committee Support Paper.pdf

Thank you for sharing this Kevin. Can you please give me high-level summary, in no more than 100 words, that describes the problem and your request? I'm not sure I'm tracking what the ask is here.

Thank you,

Deb

From: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Sent: Thursday, February 29, 2024 3:13 PM
To: Ward Lund, Debra L. (IHS/PHX) <Debra.Wardlund@ihs.gov>
Subject: Policy Committee Support Paper.pdf

Deb,

Since you detailed me to this "***mission-critical position,"*** I've highlighted the alarming state of our policy system and processes. The Joint Commission Resources mock survey exposed the dire consequences, confirming the danger our patients face due to outdated and inaccurate policies. This failure fundamentally undermines our mission and jeopardizes our Joint Commission readiness. I have tried to stay within my Chain of Command. However, there are situations where breaking the chain of command is not only warranted but absolutely necessary. This is one of those times, as it involves patient safety. Though I have been told I have the full support of my chain of command, I've found little substantive discussion or support for the necessary changes during broader meetings. This lack of engagement further delays and undermines the urgency to protect our patients and uphold our mission.

Despite repeated efforts, including communication through my chain of command, I'm met with responses claiming that policy review is the responsibility of existing committees like MEC, Nursing, and Pharmacy. However, I've found glaring evidence that this isn't working effectively. Numerous recently "updated" policies still contain outdated references and rely on evidence-based materials that

are sometimes ***decades*** old. Outdated references and evidence-based materials in policies create a direct risk to patient safety and undermine the ability of the Phoenix Indian Medical Center to fulfill its mission of providing quality care. It's crucial to prioritize regular policy reviews and incorporate the latest evidence to protect both patients and the institution's reputation

I have also received responses from my chain, like "We need to gather data" or "We have to pick our battles," demonstrating an unacceptable lack of urgency when patient safety and Joint Commission readiness are at stake. Every day, we hesitate and put more patients at risk.

I have attached the following:

- An executive summary outlining the critical risks to patient safety identified in the survey.
- An SBAR analysis with background, consequences, and proposed solutions.
- An Evidence-Based white paper supporting the recommendation.

The Joint Commission survey is imminent, and we cannot claim to "do what it takes to care for our patients" if we don't urgently address this crisis. We need a dedicated policy committee *and/or* a radical overhaul of our committee policy approval system – delay puts more patients in harm's way. We have a moral and professional obligation to demonstrate our unwavering commitment to patient care and compliance with Joint Commission standards.

It is disheartening to consistently hear you mandate to "do what it takes to care for our patients" yet see little urgency from others in leadership on this critical issue. With the Joint Commission survey looming, we cannot afford inaction. We must demonstrate a clear plan and progress toward correcting these deficiencies.

Deb, I would like you to take a look at the attached material. If you agree something needs to be done, I would like you to leverage your leadership to overcome this resistance and prioritize patient safety. Every moment wasted is a moment our patients are at risk. We must act decisively to demonstrate a commitment to patient care and a readiness to meet Joint Commission standards.

Thank you for taking the time to look into this critical matter.

Sincerely,

Hon. Kevin D. Turnage, MSN Ed., RN, AMB-BC™, NE-BC®, NHDP-BC™, NPD-BC™
Policy Officer (Acting), COR II,
Phoenix Indian Medical Center
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